MIZORAM PUBLIC SERVICE COMMISSION

TECHNICAL COMPETITIVE EXAMINATION FOR JUNIOR GRADE OF MIZORAM HEALTH SERVICES (COMBINED) (GDMO SUB-CADRE) UNDER HEALTH & FAMILY WELFARE DEPARTMENT, GOVERNMENT OF MIZORAM. (SEPTEMBER-2025)

TECHNICAL SUBJECT PAPER-II (ALLOPATHY)

Time Allowed : 2 hours FM : 200

All questions carry equal marks of 2 each.
Attempt all questions.

1. Different phases of Perioperative care include all of the followings EXCEPT:
(a) Postoperative care (b) Palliative care
(c) Preoperative care (d) Follow-up care

2. Liver function test include all of the following EXCEPT:
(a) Serum Bilirubin (b) Serum Creatinine
(c) Serum Albumin (d) Serum Alkaline phosphatase

3. Anchovy sauce pus is a feature of:
(a) Amoebic liver abscess (b) Lung abscess
(c) Splenic abscess (d) Pancreatic abscess

4. Not included in Charcot's triad:
(a) Fever (b) Pain
(c) Jaundice (d) Gall Bladder Lump

5. C-Reactive Protein (CRP) is an indicator of:
(a) Pyrexia (b) Analgesia
(c) Inflammation (d) Myalgia

6. Not included for pre-operative check-list before operation-
(a) Patient's name (b) Consent-mark side
(c) All investigations available (d) Social status

7. Gas gangrene is caused by:
(a) Clostridium perfringens (b) Clostridium botulinum
(c) Clostridium difficile (d) Clostridium tetani

8. A patient presents in emergency with a cervical spine fracture. First thing to do is:
(a) Locate the fracture by shifting the patient side to side
(b) X-ray of spine
(c) Clear the airway and intubate him
(d) Immobilize the cervical spine

9. After head injury, biconvex, lenticular shape hematoma in CT scan is characteristic of which of the following:
(a) Extradural hemorrhage (b) Subdural hemorrhage
(c) Intracerebral hematoma (d) Diffuse axonal injury

10. Management for CSF rhinorrhea is:
(a) Plain x-ray and packing of nose
(b) Nasal packing only
(c) Antibiotics and observation
(d) Immediate surgery

11. After rupture of middle meningeal artery bleeding occurs in which region:
(a) Subdural bleed
(b) Extradural bleed
(c) Intracerebral bleed
(d) Subarachnoid bleed

12. First step taken in case of multiple injuries of face & neck:
(a) Blood transfusion
(b) IV fluids
(c) Reconstruction
(d) Maintenance of airway

13. Best prognostic factor for head injury is:
(a) Glasgow coma scale
(b) Age
(c) Mode of injury
(d) CT Scan

14. In severe injury, first to be maintained is:
(a) Hypotension
(b) Dehydration
(c) Airway
(d) Cardiac status

15. Ulcer that may develop in burn tissue is:
(a) Marjolin's
(b) Rodent
(c) Melanoma
(d) Curlings

16. Which of these does not change or remains same throughout life:
(a) Salmon patch
(b) Strawberry angiomas
(c) Portwine stain
(d) Capillary hemangiomas

17. Late death in burns is due to:
(a) Sepsis
(b) Hypovolemia
(c) Contractures
(d) Neurogenic

18. What is the most important aspect of management of burn injury in the first 24 hrs?
(a) Fluid resuscitation
(b) Dressing
(c) Escharotomy
(d) Antibiotics

19. Most common abdominal organ injured in blunt trauma abdomen is:
(a) Spleen
(b) Liver
(c) Pancreas
(d) Stomach

20. Preferred incision for abdominal exploration in blunt trauma abdomen is:
(a) Always midline incision
(b) Depending upon the organ
(c) Transverse incision
(d) Paramedian

21. Commonest site of peptic ulcer perforation:
(a) Anterior aspect of the 1st part of duodenum
(b) Posterior aspect of the 1st part of duodenum
(c) Greater curvature of the stomach
(d) Anterior aspect of 2nd part of duodenum

22. Normal portal vein pressure is:
(a) <3 mmHg
(b) 3-5 mmHg
(c) 5-10 mmHg
(d) 10-12 mmHg

23. A 25 year old man presents with 3 days history of pain in the right lower abdomen and vomiting. Patient's general condition is satisfactory and clinical examination reveals a tender lump in the right iliac fossa. The most appropriate management in this case would be:
(a) Immediate appendectomy
(b) Exploratory laparotomy
(c) Oschner Sherren regimen
(d) External drainage

24. Most common site of hypertrophied keloid is:
(a) Face (b) Leg
(c) Presternal Area (d) Arm

25. Congenital hydrocele is best treated by:
(a) Eversion of sac (b) Excision of sac
(c) Lords procedure (d) Herniotomy

26. What is the most common position of the appendix?
(a) Pelvic (b) Paracolic
(c) Retrocaecal (d) Retroperitoneal

27. Which of the following Vitamin deficiencies is seen in short bowel syndrome with ileal resection?
(a) Vitamin K (b) Vitamin B 12
(c) Vitamin B1 (d) Folic acid

28. Referred pain to the shoulder in case of acute cholecystitis is:
(a) Murphy's sign (b) Grey Turner sign
(c) Boa's sign (d) Cullen's sign

29. Ileal obstruction due to roundworm obstruction treatment is:
(a) Resection with end-to-end anastomosis
(b) Resection with side-to-side anastomosis
(c) Enterotomy removal of worms and primary closure
(d) Diversion

30. The burn area appears red, has blisters, and is very painful; the burn is categorised as:
(a) Superficial (b) Partial-thickness superficial
(c) Partial-thickness deep (d) Full thickness

31. In a surgical patient, malnutrition is best assessed by:
(a) Hb Level (b) Mid-arm circumference
(c) Serum albumin (d) Triceps skin fold thickness

32. The most common site for Curling's ulcer is:
(a) Ileum (b) Stomach
(c) Duodenum (d) Oesophagus

33. The thickest skin in the human body is located on the:
(a) Thighs (b) Buttocks
(c) Scalp (d) Palms and soles

34. On the human body, the thinnest skin is located on the:
(a) Chin (b) Eyelids
(c) Post auricular region (d) Neck

35. A solid mass in the skin that could be soft or hard, fixed or freely movable, and may be elevated or deep is called a:
(a) Crust (b) Callus
(c) Tumour (d) Wheal

36. Upward movement of the thyroid gland is prevented due to:
(a) Berry ligament (b) Pretracheal fascia
(c) Sternothyroid muscle (d) Thyrohyoid membrane

37. Nerves of the brachial arch are derived from:
(a) Mesoderm (b) Endoderm
(c) Neural crest (d) Neuroectoderm

38. The most typical type of anal carcinoma is:
(a) Squamous cell carcinoma (b) Adenocarcinoma
(c) Adenoacanthoma (d) Papillary type

39. Rolled up omentum is seen in the case of:
(a) Malrotation of the gut (b) Perforation peritonitis
(c) Peritoneal metastasis (d) Peritoneal tuberculosis

40. Sialolithiasis is common in which gland?
(a) Parotid (b) Sublingual
(c) Submandibular (d) Minor salivary glands

41. Which structure is not preserved in modified radical mastectomy?
(a) Cephalic vein (b) Pectoralis minor
(c) Pectoralis major (d) Branches of the intercostobrachial N.

42. Most crucial diagnostic feature of congenital hypertrophic pyloric stenosis:
(a) Metabolic alkalosis (b) Non-bilious vomiting
(c) Jaundice (d) Fever

43. For recurrent anal fistula, the most appropriate investigation is:
(a) Endorectal US (b) Colonoscopy
(c) MRI (d) Proctoscopy

44. Choledochal cyst is a dilatation of:
(a) Gall bladder (b) Hepatic artery
(c) Hepatic cyst (d) Bile duct

45. What is intussuscepiens?
(a) The entire complex intussusception (b) The entering layer
(c) The outer layer (d) The process of reducing the intussusception

46. Drainage of cervical abscess is an example of:
(a) Clean contaminated wound (b) Clean an uncontaminated wound
(c) Unclean, uncontaminated wound (d) Dirty infected wound

47. Most tissue reaction is seen in:
(a) Plain catgut (b) Silk
(c) Polydioxanone (d) Chromic catgut

48. In the case of females, the commonest hernia is:
(a) Direct inguinal hernia (b) Femoral hernia
(c) Indirect inguinal hernia (d) Incisional hernia

49. A 65-year-old female presents with a swelling in the neck, diagnosed as a solitary thyroid nodule. The patient undergoes investigation, and a scan reveals increased iodine uptake. Serum T3 and T4 are elevated. The most probable diagnosis is:
(a) Benign colloid goitre (b) Follicular adenoma
(c) Toxic adenoma (d) Toxic multinodular goitre

50. Neurosurgery is indicated in all, except:
(a) SDH (b) EDH
(c) Intracranial bleed (d) Diffuse axonal injury

51. On examination, a primigravida who is 7 weeks amenorrhoeic is found to have bluish coloration of vaginal mucosa. This sign suggestive of pregnancy is called:

(a) Hegar's sign
(b) Palmer's sign
(c) Goodel's sign
(d) Jacquemiers sign

52. If on per vaginal examination, you find that the sagittal sutures are irreducible and overlapping. What is the grade of moulding here?

(a) Grade 1
(b) Grade 2
(c) Grade 3
(d) Grade 4

53. On clinical examination per vaginally, you find that ala of sacrum is absent, what is this type of pelvis called?

(a) Robert's pelvis
(b) Rachitic pelvis
(c) Android pelvis
(d) Osteomyelitic pelvis

54. What is the best diagnostic test for iron deficiency in pregnancy?

(a) Serum iron level
(b) Transferrin saturation
(c) Total iron binding capacity
(d) Serum Ferritin

55. When is the blood volume maximum in pregnancy?

(a) 34th weeks
(b) 36th weeks
(c) 32nd weeks
(d) 28th weeks

56. All of the following maternal causes can lead to fetal tachycardia except:

(a) Pyrexia
(b) Dehydration
(c) Hypothyroidism
(d) Hypovolemia

57. Most common chromosomal anomaly in abortion is:

(a) Monosomy
(b) Trisomy
(c) Translocation
(d) Triploidy

58. A G4P3 female patient who is agricultural worker presents with fatigue at 38 weeks of gestation. On examination, pallor, mild pedal edema is seen . Her Hemoglobin is 6.8gm%. Name the appropriate treatment:

(a) Iron sucrose infusion after calculating total dose required
(b) Ferric carboxymaltose injection
(c) Packed RBC transfusion
(d) Iron dextran

59. Site for maximum absorption of iron is:

(a) Duodenum
(b) Jejunum
(c) Ileum
(d) Stomach

60. One 300mcg dose of anti D will neutralize how much of fetal blood?

(a) 15 ml of fetal red cells
(b) 40 ml of fetal whole blood
(c) 20 ml of fetal red cells
(d) 15 ml of fetal whole blood

61. The most common organism responsible for Asymptomatic bacteriuria in pregnancy is:

(a) Klebsiella
(b) E coli
(c) Pseudomonas
(d) Proteus

62. What is the risk of vertical transmission of HIV during pregnancy and delivery and breastfeeding without ART?

(a) <10%
(b) 10-30%
(c) 30-50%
(d) >50%

63. Which of the malarial drugs is safest in pregnancy?
(a) Chloramphenicol
(b) Quinine
(c) Chloroquine
(d) Primaquine

64. Consumptive coagulopathy is more likely with:
(a) Revealed abruption
(b) Placenta praevia
(c) Concealed abruption
(d) Grade 1 abruptio placentae

65. What is the treatment of red degeneration in pregnancy?
(a) Myomectomy
(b) Conservative
(c) Termination of pregnancy by medical induction
(d) Early artificial rupture of membrane

66. The triple test in second trimester screening includes all except:
(a) Alpha fetoprotein
(b) Estradiol
(c) βHcg
(d) Inhibin A

67. The upper limit of age of gestation for medical termination of pregnancy by one registered practitioner is:
(a) 12 weeks
(b) 20 weeks
(c) 8 weeks
(d) 16 weeks

68. Which of the following is not a special measure for a woman with diabetes on insulin during childbirth?
(a) The morning dose of insulin to be withheld
(b) Start monitoring blood glucose level
(c) IV Normal saline to be started and insulin given according to blood glucose level
(d) Give the usual medication on the day as before

69. What dose of misoprostol is used for induction of labour?
(a) 50μg
(b) 25μg
(c) 100μg
(d) 200μg

70. Time interval between alert line and active line in partograph is:
(a) 6 hours
(b) 5 hours
(c) 4 hours
(d) 10 hours

71. Which of the following condition is at high risk due to an increased level of estrogen in PCOS?
(a) Hirsutism
(b) Endometrial cancer
(c) Sleep apnoea
(d) Dyslipidemia

72. Which ovarian tumour can cause thyrotoxicosis due to functioning thyroid tissue?
(a) Carcinoid tumour
(b) Theca lutein cyst
(c) Mucinous cystadenoma
(d) Struma ovarii

73. A multigravida is in second stage of labour for 2 hours. Per vaginal examination show station at 0 with caput and +2 Moulding. Sagittal suture in right occipito transverse. Foetal heart rate is 140. Appropriate management will be:
(a) Emergency LSCS
(b) Mid pelvic Forceps delivery
(c) Fundal pressure
(d) Vacuum delivery

74. Which manoeuvre is not included in the management of shoulder dystocia?
(a) Mc Robert's
(b) Zavanelli
(c) Fundal pressure
(d) Wood's screw manoeuvre

75. A 30 year-old pregnant woman has a history of mitral valve replacement. She is on warfarin. At 6 weeks pregnancy, what is the best management?
(a) Continue warfarin as before
(b) Switch to LMWH unit until 12 weeks , then resume warfarin
(c) Switch to aspirin
(d) Stop all anticoagulants

76. In "Anemia Mukt Bharat operational guideline 2018" What is the recommended oral iron and folic acid supplementation in pregnancy by The Ministries of Health, Govt of India?
(a) Daily 100mg of elemental iron with 500 μg of folic acid from 14 weeks onwards for 100 days
(b) 60 mg of elemental iron with 400 mg of folic acid daily for latter 6 months of pregnancy till 3 months post-partum
(c) 60 mg of elemental iron plus 500 μg of folic acid daily starting from 2nd trimester for 180 days and to be continued for 180 days after delivery
(d) 100 mg of elemental iron plus 400 μg of folic acid daily for latter 6 months of pregnancy till 6 months post-partum

77. Which of the following test are included in Fetal Biophysical profile?
(a) NST, Liquor volume, Fetal tone, Fetal breathing movement and fetal movements
(b) NST and Liquor volume
(c) NST, fetal breathing movements, fetal position, Fetal tone and doppler study of umbilical vein
(d) Doppler study of umbilical, MCA, Ductus arteriosus and Uterine arteries

78. Which vaccination is not contraindicated during pregnancy?
(a) MMR
(b) Varicella
(c) Influenza
(d) HPV

79. A sudden bleeding per vagina at 37 weeks which is painless, bright red. On examination, patient is pale, BP 80/50mmHg, pulse 125/min, and presenting part is unengaged. Management should be:
(a) Immediate resuscitation and termination of pregnancy by LSCS
(b) Immediate resuscitation and wait till due date
(c) Perform artificial rupture of membrane and try vaginal delivery
(d) Do a double set up Pelvic examination at OT after resuscitation

80. A grand multipara complains of scanty, painless, bleeding per vagina at 38 weeks of pregnancy. On examination, patient is stable, bleeding is not copious, but there is fetal heart deceleration. The likely cause of bleeding is:
(a) Abruptio placentae
(b) Vasa previa
(c) Placenta previa
(d) Combined Abruption with placenta previa

81. Symptoms of anemia in pregnancy are all except:
(a) Loss of appetite
(b) Headache
(c) Shortness of breath
(d) Leucorrhoea

82. Which of the following is not seen in impending eclampsia?
(a) Blurring of vision
(b) Occipital Headache
(c) Epigastric pain
(d) Lower abdominal pain

83. The Pathophysiology of pre-eclampsia is mainly due to:
(a) Abnormal trophoblastic invasion and remodeling of spiral arterioles
(b) Increased level of estrogen and progesterone
(c) Increased level of aldosterone
(d) Decrease in blood volume

84. Which of the following congenital anomaly is most commonly seen in pre gestational diabetes?
(a) Cleft lip
(b) Cardiac anomalies
(c) Renal agenesis
(d) Anencephaly

85. In a patient with intrahepatic cholestasis of pregnancy, which is the drug of choice for symptom control and improving fetal outcome?
(a) Cholestyramine
(b) Ursodeoxycholic acid
(c) Prednisolone
(d) Lactocalamine

86. A pregnant woman in her first trimester is diagnosed with Primary syphilis. What is the choice of treatment?
(a) Azithromycin
(b) Benzathine Penicillin
(c) Doxycycline
(d) Ceftriaxone

87. Which of the following infections causes sensori-neural deafness, especially if contracted in the first trimester?
(a) Rubella
(b) Toxoplasmosis
(c) HSV
(d) Varicella

88. 25 years old, 10 weeks pregnant woman was found to have 7 cm ovarian cyst with thin wall and clear fluid on routine ultrasound. Next action would be:
(a) Immediate Surgery
(b) MRI Pelvis
(c) Start progesterone support
(d) Repeat scan after 6 weeks

89. A 35 year old woman in early pregnancy presents with a large, obstructing cervical fibroid. What complication is likely during labor?
(a) Uterine rupture
(b) Placenta previa
(c) Obstructed labor
(d) Preeclampsia

90. Which of the following diameters is considered most important in assessing pelvic inlet adequacy?
(a) True conjugate
(b) Transverse diameter
(c) Diagonal conjugate
(d) Obstetric conjugate

91. In which diameter of the presenting part is vaginal delivery impossible?
(a) Occipito -frontal
(b) Sub Occipito frontal
(c) Sub mento Bregmatic
(d) Mento-vertical

92. Which of the following agents is safest for cervical ripening in a case of previous caesarean section?
(a) Misoprostol
(b) Dinoprostone gel
(c) Foleys intracervical catheter
(d) Mifepristone

93. Abnormal uterine bleeding in adolescent is most commonly due to;
(a) Anovulation
(b) Bleeding disorders
(c) Endometrial carcinoma
(d) Adenomyosis

94. The most common malignant ovarian tumour in young women under 30 years is:
(a) Serous Cystadenoma
(b) Dysgerminoma
(c) Granulosa cell tumour
(d) Immature teratoma

95. All are Rotterdam criteria for diagnosing PCOS except:
(a) Oligo-or anovulation
(b) Hyperandrogenism
(c) Elevated LH/FSH
(d) Polycystic ovaries on ultrasound

96. The most common causative organism in PID are:
(a) E coli and klebsiella
(b) Chlamydia trachomatis and Neisseria Gonorrhoeae
(c) Mycoplasma and ureaplasma
(d) Gardenella Vaginalis and Candida

97. A 36 year old multipara presents at 8 weeks with bleeding per vagina and lower abdominal pain.On examination, Os is closed, uterus is 8 weeks size and foetal heartbeat is present on USG. What is the likely diagnosis?
(a) Complete abortion
(b) Threatened abortion
(c) Missed abortion
(d) Inevitable abortion

98. A 16 years old girl is brought to the clinic with a complaint of fever, foul smelling vaginal discharge, hypotension and bleeding after MTP done illegally 3 days back. Immediate step of management would be:
(a) USG to confirm retained products
(b) Stabilise and start broad spectrum antibiotics
(c) Start misoprostol
(d) Schedule evacuation immediately

99. Which of the following is most consistent with uterine perforation during surgical treatment of abortion?
(a) Abdominal pain
(b) Loss of resistance
(c) Bradycardia
(d) Bleeding

100. A woman presents with bleeding per vagina with severe abdominal cramps. Ultrasound show empty uterine cavity. Serum Beta HCG is 1500IU/L. Which is the most likely diagnosis?
(a) Complete abortion
(b) Threatened abortion
(c) Missed abortion
(d) Ectopic pregnancy

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